

Office Visit — January 21, 2025

with Miriam Okafor-Stein, MD at Riverside Otolaryngology Associates, 88 Deverell Avenue, Suite 410, Brooklyn, NY 11205 · (718) 555-0164

Notes from Care Team

Progress Notes

Miriam Okafor-Stein, MD · Otolaryngology · Electronically signed 01/21/2025 11:05 AM

RIVERSIDE OTOLARYNGOLOGY ASSOCIATES DIVISION OF RHINOLOGY & SINUS SURGERY

DOS: January 21, 2025
Name: Theo Marchbank
DOB: 1994-03-12
Address: Apt 3B, 412 Calver Street, Brooklyn, NY 11205
Phone: (212) 555-0147

Visit Type: Established Patient Office Visit — Follow-up
Referring Provider: Dana Kellerman, MD

Reason for Visit:

- Follow-up: septal deviation and recurrent rhinosinusitis, on medical therapy
- “The spray and rinses are helping — I can breathe through the left side most nights now. One mild infection over the holidays but it cleared on its own.”*

History of Present Illness:

30 y.o. male returns for planned 3-month follow-up of leftward septal deviation with posterior spur and bilateral inferior turbinate hypertrophy documented on endoscopy at his initial visit on 10/14/2024. Since starting daily intranasal fluticasone and saline irrigation he reports meaningful improvement: left-sided obstruction now intermittent rather than constant, snoring reduced per partner, nighttime mouth breathing largely resolved.

One URI-associated sinus episode in late December, milder than prior episodes, resolved without antibiotics in about a week. Compliant with sprays; has stopped OTC decongestant entirely.

Symptom — Nasal congestion: Yes — improved, intermittent
Symptom — Facial pain or pressure: No
Symptom — Post-nasal drainage: Occasional
Symptom — Decreased sense of smell: No
Symptom — Nosebleeds: No — minor crusting only
Symptom — Snoring: Improved

ALLERGIES: No known drug allergies on file

MEDICATIONS:

- fluticasone propionate (generic) 50 mcg/actuation nasal spray — 2 sprays each nostril daily; Disp: 16 g; Rfl: 3 — CONTINUED
- saline nasal irrigation, large volume — daily, CONTINUED

VITALS:
BP: 121/76
Pulse: 70

Weight: 171 lb (77.6 kg)

BMI: 23.8

PHYSICAL EXAMINATION:

General: Well-appearing, no acute distress.

Nose/anterior rhinoscopy: Septal deviation to the LEFT unchanged, as expected; turbinate mucosa notably less edematous than at the 10/14/2024 visit. Improved left-sided airflow on inspection. No purulence, no significant crusting.

Oral cavity/oropharynx: Unremarkable.

Impression/Summary:

1. **Deviated nasal septum (leftward, with posterior spur)** — anatomically unchanged; symptoms improving on medical therapy
2. **Recurrent acute rhinosinusitis** — one mild self-limited episode since last visit — improved trajectory
3. **Inferior turbinate hypertrophy, bilateral** — reduced mucosal edema on today's exam

Plan/Recommendations:

1. Continue intranasal fluticasone and daily saline irrigation; good symptomatic response, so no imaging indicated at this time.
2. Deferred septoplasty: given mild residual symptoms and improvement on medical therapy, patient elects to continue conservative management. Revisit surgical discussion if obstruction worsens or infection frequency rebounds.
3. Counseled that anatomic deviation will not change with medication; symptom control is the treatment goal.

Follow up: Return as needed; recommend re-evaluation if more than two sinus infections in the next 12 months. Records available to patient via portal for overseas relocation.